STANDARD OPERATING PROCEDURE			P-9400	
	SARPY COUNTY SHERIFF'S OFFICE PAPILLION * NEBRASKA GREG LONDON SHERIFF	DATE ISSUED 03/24/21	DATE REVISED **07/29/22**	DATE REVIEWED 02/02/25
		APPROVED: *GREG LONDON*		
CHAPTER TITLE ROAD PATROL	RESOURCES			
SUBJECT MHCSU OPERATIONS		CROSS REFERENCE		

I. Purpose:

The purpose of this policy is to establish guidelines for the operations of the Mental Health Crisis Support Unit.

II. Policy:

Deputies assigned to the Mental Health Crisis Support Unit (MHCSU) will assist individuals in attempting to restore them to a pre-crisis level. The goal is to de-escalate situations safely for all individuals when reasonable, practical, and consistent with established safety priorities. Responding to situations involving individuals who reasonably believe to be affected by mental illness or in crisis carries the potential for violence. Only trained mental health professionals can diagnose mental illness, and even they may sometimes find it difficult to make a diagnosis. MHCSU members are not expected to diagnose mental or emotional conditions but are expected to recognize behaviors that are indicative of persons affected by mental illness, with a special emphasis on those that suggest potential violence and or danger. This requires a MHCSU deputy to make difficult judgements about the mental state and intent of the individual. MHCSU deputies will use their training and skills in an attempt to better handle situations where mentally ill consumers come in contact with law enforcement.

III. Procedure:

A. MHCSU Dress Code: MHCSU deputies may wear the standard road patrol uniform (see G-3310 Road Patrol Uniforms) or they may wear civilian attire adhering to G-3200 Dress and Attire with the following additions.

1. MHCSU deputies may wear tactical pants (color optional).

2. MHCSU deputies may wear reasonable attire depending on weather conditions. This may include polo shirts, sweatshirts and/or pullover jackets.

Garments shall be adorned with the Sheriff logo or MHCSU logo. An approved Sarpy County Sheriff's Office Specialty Unit shirt may also be worn.

3. MHCSU deputies shall have available a black tactical vest carrier with "SHERIFF" prominently displayed on the front and back.

B. **Response:** The MHCSU may be self-dispatched or requested by a supervisor. If requested by a supervisor the MHCSU deputy shall be briefed on call type, familiarity of person(s) involved, and any relevant details related to the call. Upon arrival and being briefed, the MHCSU deputy may relieve an assigned unit who may return to service if situation allows. At least two deputies should be present on all mental health calls for service involving persons with a mental illness. A member of the MHCSU and an initial responding sworn deputy/officer shall meet criteria for a two-deputy response. The MHCSU deputy shall have discretion in this area depending on the person in crisis and call for service.
The MHCSU shall not provide transportation or assist other agencies with mental health related calls for service, without approval from either the MHCSU Supervisor or an SCSO command officer.

C. **Deputy Responsibilities:**

1. All Deputies regardless of assigned division shall complete a Crisis Intervention Tracking form for calls involving a person having a mental illness, regardless of the initial call type, contact, and disposition. These forms shall be forwarded to the MHCSU for review and follow up. The MHCSU supervisor shall track and document all contacts, calls for service, and tracking forms forwarded to the unit. The assigned deputy on any call for service which may involve a person with mental illness shall forward relevant reports to the MHCSU for possible follow up. Deputies assigned to the MHCSU may attempt to contact persons forwarded to the unit to offer assistance coordinated with a therapist from ASAP. Deputies assigned to the MHCSU shall work closely with the Sarpy County Mental Health Program Attorney and the Heartland Family Service Crisis Response Services (ASAP) Director.

2. The assigned deputy regardless of division shall be responsible for involuntary committals (EPC's) involving a call for service where a member of the MHCSU is not on scene. The MHCSU shall not respond to calls for service involving involuntary committals after regular duty hours unless requested by a command officer. The assigned deputy shall handle and complete all reports and paperwork related to the initial call for service and forward all relevant reports and information to the MHCSU. Calls for service involving perceived mental health issues **shall not be held** for the MHCSU if the unit is engaged in other assignments or unavailable.

3. Some mental health related calls for service are outside the scope of the MHCSU training. Among these calls are active suicide attempts, standoffs, barricaded subjects, and hostage takings. The MHCSU is not a substitute for the Crisis Negotiations Unit, SWAT, or the Warrants Unit. A member of the MHCSU may be requested by a command officer to be on scene for call outs involving CNU and ESU.

D. Response for Persons with a Mental Illness:

1. Deputies should be prepared to encounter a person with a mental illness at any time.

2. Persons with mental illness can be easily upset and may engage in tantrums or self-destructive behavior, minor changes in daily routines may trigger these behaviors.

3. Frequently, a family member or friend is of great value in calming an individual exhibiting unusual behavior such as those resulting from mental, intellectual, or emotional impairment.

4. The following guidelines detail how to approach and interact with persons who may have mental illness while protecting the deputy's own safety, the safety of the person with mental illness, and others at the scene:

a) Speak calmly; loud, stern tones will likely have either no effect or a negative effect on the individual.

b) Use non-threatening body language; unless the situation requires an aggressive posture, the deputy's hands should be kept free of weapons, Taser, OC spray, and handcuffs. Use open hand movements and avoid quick-rapid motion.

c) Eliminate commotion; eliminate to the degree possible, loud sounds, bright lights, sirens, and crowds, moving the individual to a calm environment, if possible.

d) Keep animals away: Individuals with mental illness are often afraid of dogs or other large animals.

e) Look for personal identification: Medical tags or cards often indicate mental illness and will supply a contact name and telephone number.

f) Call the caregiver: The caregiver is often the best resource for specific advice on calming the person and ensuring both the officer's safety and the person's safety until the contact person arrives.

g) Prepare for a lengthy interaction: Mentally ill individuals should not be rushed unless there is an emergency.

h) Use short, direct phrases: Too much talking can distract the mentally ill individual and confuse the situation.

i) Be attentive to sensory impairments:

(1) Many mentally ill individuals have sensory impairments that make it difficult to process information. Deputies should not touch the person unless absolutely necessary, use soft tones & gestures, avoid quick movements, use simple and direct language, and don't automatically interpret odd behavior as belligerence.

j) Once sufficient information has been collected about the nature of the situation, and the situation has been stabilized, there is a range of options deputies should consider when selecting an appropriate resolution. These options include the following:

(1) Refer or arrange transport for the person if medical attention if person is Injured. If abuse is occurring, investigations shall be notified and requested.

(2) Outright release, if no criminal activity has occurred and the subject is stable, does not present a danger to themselves or others and is not an EPC candidate.

(3) Release to care of family, caregiver or mental health provider.

(4) Refer person to substance abuse services.

(5) Assist in arranging voluntary admission to a mental health facility if requested.

(6) Arrange transport for involuntary emergency psychiatric evaluation if the person's behavior meets the criteria for this action.

(7) Arrest if a crime has been committed.

(8) Contact the Mobile Crisis Team (ASAP) for assistance.

E. Custody:

1. If an individual with a mental, emotional, or psychological illness is taken into custody, deputies shall make a responsible effort to use the least restraint possible and protect the arrestee from self-injury, while taking all necessary precautions. The overall circumstances and the person's potential for violence will determine if handcuffs will be secured in the front or behind the

back of the subject as a temporary measure to prevent injury to the individual or deputy, adhering to established policies and procedures.

2. In a misdemeanor incident where an individual is apparently mentally ill, deputies should seek non-arrest resolutions. The most desired resolution being voluntary admission to an appropriate mental health facility. However when public safety is at issue, deputies shall follow Sheriff's Office policy and procedure regarding custodial arrests, cite and releasing subjects and involuntary emergency protective custody (EPC).

F. **Voluntary Admission:** The three following scenarios would indicate minimal deputy involvement:

1. Persons who appear to be in need of psychiatric evaluation and do not appear to pose an imminent danger to themselves or others should be referred to a mental health facility. (A family member or other responsible person is often available to assist the disturbed person in seeking such treatment and should be provided with the information necessary to secure the needed help.) Mobile Crisis could also be requested to assist in this case.

2. Persons who have been or are under the care of a private physician should be referred to the physician if possible.

3. Persons, who voluntarily agree to psychiatric evaluation, will be taken to the closest hospital, or transported to a mental health crisis center.

G. **Involuntary Admission (EPC):** A higher level of law enforcement intervention will be required when officers encounter the following scenarios:

1. The person is imminently dangerous to self or others. This includes threats of suicide and attempted suicides, threats against others or homicidal ideations.

2. The person is unable to care for self (unable or refuses to accept intervention which would meet needs for food, clothes, prescribed medications, shelter, or physical well-being.)

3. The person is suffering substantial physical deterioration and shows an inability to function if not treated immediately.